

Policy & Procedure of Informed Consent



Ministry of Health

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1. Introduction:

The Informed Consent Policy and Procedure are the main legal reference for the Ministry of Health in Oman as a matter of good practice that ensures patients' safety and ongoing communication between patients and healthcare providers. Therefore, all healthcare institutions must replace their local policies, procedures, and guidelines to conform to the national informed consent policy. This policy is compulsory and the MOH mandates all healthcare providers to ensure compliance with the legally and ethical standards outlined within this policy, in obtaining Informed Consent from patient/legal authorized representatives. Informed Consent for any form of research will NOT be covered under this policy.

By implementing this policy, all healthcare providers are obligated to:

- Abide with legal, professional, and ethical obligations in obtaining informed consent prior to managing their patients' medical condition/s.

- Provide sufficient information to the patient/legal representative in order to help them make informed voluntary decisions whereby they either accept or refuse the proposed surgery/treatment or procedure.
- Respect the right of the patient/legal representative to refuse the offered treatment or to withdraw their consent as per the National Public Health Patients' Bill of Rights and Responsibilities (please refer to the Ministry of Public Health Patients' Bill of Rights and Responsibilities).
- Respect the patients' cultural requirements and fulfilled them whenever possible.

2. Purpose

To provide a comprehensive guideline on implementing the informed consent processes and protect patients and avoid putting them in powerless and vulnerable situations, as well healthcare providers.

3. Scope

This policy and procedure apply to:

- All health care institutions with inpatient and outpatient services.
- All healthcare professionals are responsible for providing informed consent.

4. Definitions

Consent: Is authorization for the proposed surgery/treatment or procedure by the patient to the health care provider.

Informed consent: This is a process in which the physician/procedure list provides adequate information for the patient or patient's legal representative to make an informed decision on the proposed treatment or procedure.

Consent form: A legal document signed or marked by a patient or legal, authorized representative voluntarily prior to any medical proposal of surgery/treatment or procedure without coercion or undue influence. This form states that the patient agrees to undergo the recommended surgery/treatment or procedure and is aware of any possible risks that might occur. The consent form can be categorized as a General Treatment Consent Form or an Informed Consent Form.

Electronic signature: Symbols or other data in the digital form attached to an electronically transmitted document verify the sender's intent to sign the document

Proceduralist: The staff who is authorized (Privileged) to perform the proposed surgery/treatment or procedure.

Witness: A witness is a person other than the proceduralist who is physically present during the discussion of the recommended treatment/procedure with the patient. A witness can be a nurse, dental assistant/technician, interpreter, patient family member, etc. and they attend the signing/marking (thumbprint) of the consent form by the patient or legally authorized representative.

Child: According to the Gregorian calendar, every human being is below the age of eighteen (Child Law, 2014). The departmental classification of patients is currently applied in the institutions of the Ministry of Health, where only patients up to 12 or 13 years old are considered children. Therefore, regardless of the patient's location in the hospital, any patient under 18 years of age should be considered ineligible for decision-making.

Competent adult: A person of 18 years of age or older with sound decision-making and communication skills who can understand and retain information regarding the nature and severity of the disease, treatment options/alternatives, and consequences.

Incompetent adult: A person at least eighteen (18) years old temporarily or permanently impaired to the extent that the person lacks sufficient understanding to make or communicate responsible personal decisions. (Which have been declared incompetent by a court of competent jurisdiction).

Legal Authorized Guardian/Representative: The authorized guardian/ representative capable of decision-making, including providing consent on behalf of a minor or incompetent adult patient.

Mental disability: A condition that limits a person's intellectual abilities, resulting directly or indirectly from injury to the brain or from abnormal neurological development.

Emergency Situation: A medical condition manifesting itself by acute symptoms of sufficient severity such that the absence of immediate medical attention is expected to result in:

- Placing the health of the individual in serious jeopardy.
- Serious impairment of body functions.
- Serious dysfunction of a body organ.

5 Policy

All patients/legal representatives have the right to make decisions regarding their healthcare and to be provided with sufficient information in order to make informed decisions.
(Oman National Patient Rights & Responsibilities Declaration phrase 4e4).

All patients/legal representatives must be given the opportunity to give "Informed Consent" prior to the administration of anesthesia by an anesthesiologist and prior to the performance of operative and/ or invasive procedures, or situations when it seemed advisable to have formal documentation of the patient's consent for proposed surgery/treatment or procedure.

5.1 There are three types of informed consent:

Implied Consent is when the patient/legal representative indicates their agreement and cooperation for certain procedures, for example, blood tests.

Written Consent (Informed Consent) is when the patient/legal representative signs the consent to confirm their agreement to the proposed surgery/treatment or procedure to protect the patient and avoid any expected risks or complications.

Verbal Consent is when a patient/legal representative verbally states the agreement to the proposed surgery/treatment or procedure, which does not involve significant risks.

5.2 Informed consent is required for the following in all the institutions

- i. Specific treatment, intervention, procedures & surgery for example:
 - a. Minor procedures such as IUCD Insertion and removal
 - b. Implanon insertion
 - c. NGT Insertion
 - d. Urinary catheter Insertion
 - e. Dental procedures for high-risk patients
 - f. Surgical cut wound suturing
 - g. I and D of abscess
 - h. Removal of Foreign body
 - i. Dressing
 - j. Dental extraction
 - k. Ear Wax removal
- ii. Blood/blood products transfusions
- iii. Giving any type of anesthesia (general, regional anesthesia, or procedural sedation).
- iv. Performing interventional exams
- v. Giving chemotherapy or radiotherapy
- i. Hemodialysis procedures.

- vii. Utilizing body parts and tissues that have been removed during operations
- viii. Using any patient-related materials for research, study, or dissemination.
- ix. In case of compulsory admission for psychiatric patients, Mental Health Care Law shall be followed.
- x. Taking a photograph of the patient for therapeutic or educational purposes. With the exception of the following situation where the consent to make the recording will be implicit in the consent given to the primary intervention and the image or its use should not be inferred from the patient's personality, either directly or indirectly. (see Appendix 1 for the consent form)
 - a. Images of internal organs or structures or external body parts.
 - b. Images of pathology slides
 - c. Laparoscopic and endoscopic images
 - d. Recordings of organ functions
 - e. Ultrasound images
 - f. X-rays
- The decision to either consent or not consent to the proposed surgery/treatment or procedure must be made by the patient or his/her legal representatives. This decision must not be influenced by anybody including the medical staff, friends, or family members.
- When the patient is deaf, dumb or blind, the treating Physician/Dentist or any other treating healthcare provider must assess the patient's ability to communicate using sight, sound, writing, sign language, or Braille language. Based on the healthcare assessment the consent must be obtained from the patient him/herself or who represents them legally.

5.3 Three elements of Informed Consent:

- a. **Disclosure:** Patients/their relatives must be adequately and appropriately informed and understand the information provided for the decision-making before obtaining consent. The healthcare provider must explain thoroughly all information on the proposed surgery/treatment or procedure, including the expected benefits and risks, and the likelihood (or probability) that the benefits and risks will occur.
- b. **Capacity and Competency:** The patient must understand the information, retain it, consider (weigh the choices), and be able to make a decision. The amount of information provided should respond to the patient's queries and his/her wishes. If the patient has no

capacity to sign the informed consent, the legal representatives must present/provide what proves the status of guardianship legally.

- c. **Voluntariness:** The relevant information must be comprehended and voluntarily grant consent without coercion, duress, or undue influence by family, friends, employers, insurers, and healthcare professionals.

- Capacity Assessment is determined by a trained physician or a psychiatrist (if applicable). The psychiatrist can be involved to assist with the capacity assessment and underlying psychiatric conditions of the patient/relative that may pose an impediment to decision-making. (See appendix. 1)
- The following group of incapacity under the legal categories, who have a significant impairment of mental abilities, are considered 'decisional incompetent.' They are not always able to determine what is in their interest:
 - i. Minority
 - ii. Unconscious patients.
 - iii. Severe mental disorder
 - iv. Mental disability
 - v. Geriatric
- Patients should participate in their treatment plan as per the "National Document of patient rights and Responsibilities," one of their fundamental rights.
- The treating Physician/Dentist/ Proceduralist or any other treating healthcare provider be accurate and impartial in providing medical information by explaining to the patient (or/ and relative) about the various treatments, medical tests, and the necessary procedures based on the patient's condition.
- A patient must be authorized to give informed consent prior to the initiation of the proposed surgery/treatment or procedure and/or invasive procedures
- The treating Physician/Dentist/ Proceduralist or any other treating healthcare providers is responsible to take and document the Informed Consent in Al-SHIFA 3+ system.
- The treating Physician/Dentist/ Proceduralist or any other treating healthcare provider is responsible to document any clinical circumstances and/or other relevant reasons for not taking the informed consent.

5.4 Validity of the Informed Consent:

- i. If there is no change in the patient's condition, the nature, extent, or reason for the proposed surgery/treatment or procedure, the patient's consent will remain valid for one year.

- ii. For each Surgical Procedure, Specific Procedures, and Therapeutic Intervention, a single Informed Consent Form must be signed, and it must be valid for the duration of the specified Treatment Plan.
- iii. If the patient revokes consent or cannot recall the information, or if the patient's medical condition, risks/benefits, treatments/procedures, and/or the alternatives to which the patient provided consent, the consent form will become invalid. A new consent must be obtained accordingly.
- iv. Blood/blood product transfusion consent is valid for the entire current admission; if the patient requires multiple blood product transfusions during admission, only one transfusion consent form is required. When a patient needs ongoing transfusion support, consent can be valid for 12 months. The consent form should indicate the expiry date of long-term consent.
- v. In case of the consent for MRI, it should be valid for two years. However, the patient shall be evaluated by the anesthesia team within 4-6 weeks prior to the MRI.
- vi. Informed Consent is generally valid indefinitely unless withdrawn by the consenting individual. However, suppose new information becomes available between when consent was sought and when the intervention was conducted regarding the proposed surgery/treatment or procedure intervention (for example, recent evidence regarding the risks or new treatment options). In that case, the informed consent shall be reconsidered; the physician must inform the patient and re-obtain their consent. (**See appendix 2**)
- vii. Electronic Consent Forms should be considered appropriate for battlefield casualties with no identification, expatriates without a guardian, and unidentified persons with unknown residences.
- viii. Standard forms must be followed for military field or civilian (relief) operations for all teams involved in military medical and medical relief operations. However, it must be approved by the supervisory and commanding authority concerned and relevant in military and civilian medical procedures.
- ix. All healthcare institutions should have a checklist of procedures/treatments that require informed consent.
- x. Any queries should be answered and explained thoroughly and, in a language, and terminology that the patient or relative understands.
- xi. Physicians should always balance their duty to respect patients' autonomy and their responsibility to advocate for patients' well-being. Therefore, a doctor has to explore the patient's motivations to refuse the proposed surgery/treatment or procedure, assess the patient's decisional capacity and risks of outpatient treatment, and discuss a safe alternative care plan that can satisfy both the patient's medical and social needs.
- xvii. The Consent form; shall be written in understandable English and Arabic language.
- xviii. There should be no abbreviations and avoid acronyms in informed consent documents.

- xix. Only the physician/procedure specialist and the anesthesiologist can provide the information, but other staff should not provide information for the informed consent.
- xx. The witness shall be present, and the attending physician must document it on the consent form.
- xxi. The healthcare providers witnessing the process of informed consent must sign as a witness.

The legal representative (if the patient is a minor or has been judged incompetent), shall be recognized for a category of individuals as follows:

- a) A first-degree family member, for example, parents, spouse, son/daughter, adult brother, etc.).
- b) Second-degree relatives, e.g., individual grandparents, grandchildren, uncles, aunts, nephews, nieces, and half-siblings).
- c) Third-degree relatives include an individual's great-grandparents, great-grandchildren, great uncles/aunts, and first cousins.
- i. An attorney-in-fact: a person authorized to act on behalf of another person through a power of attorney document for healthcare decisions or official transactions.
- vii. An interpreter must be involved in the consent process if the patient or legal representative, is not able to read or understand either English or Arabic. The interpreter, as a witness, must sign the consent form as well.
- viii. Upon signing the consent form, if the patient or the legal representative is unable to write, (a thumbprint in the signature area will be sufficient and serve as proof of approval. The treating Physician/Dentist/ Proceduralist or any other treating healthcare providers must print the patient's name below the thumbprint.
- ix. The anesthesiologist must obtain an informed consent from the patient prior to administering anesthesia, and performing operation, or invasive procedures.
- x. The treating Physician/Dentist/ Proceduralist or any other treating healthcare providers should reassure and confirm before starting the procedure; that the patient's consent is valid.
- xi. Hospital administrations should develop strategies to reduce the likelihood of a patient's refusal of the proposed surgery/treatment or procedure, such as improving physician-patient communication, promoting shared decision-making practices, monitoring staff adherence to relevant guidelines and policies, etc.
- xii. It is recommended to give the patient relevant information including expected complications leaflet if available.

5.5 Ethical consideration in obtaining Informed Consent:

- i. Confidentiality- Do not reveal any patient information/identity.
- ii. Privacy- Do not violate patient privacy.

- iii. Clarity-Deliver the information professionally and suitably using simple language, and not using incomprehensible medical terms.
- iv. Beneficence- make sure that the patient understands the medical procedure by explaining the proposed treatment, the plan, and the options available.
- v. Autonomy- Inform the patient of the importance of his opinion and that he is a partner of the treatment plan.
- vi. Avoid the attendance of unwanted companions by the patient during the meeting.
- vii. Reliability and safety- Not to exaggerate or underestimate the expected complications and the benefits of the proposed surgery/treatment or procedure.
- viii. Autonomy- Allow the patient to take any decision instead of enforcing him.
- ix. Accountability- Make sure that the medical procedure matches the information that the patient signed upon on the informed consent

5.6 Procedure:

Elements of the Informed Consent

- i. Physician, must have all significant medical information that is relevant to make informed decision by the patient
- ii. Procedure information should be specifically listed including the following:
 - a. Detailed information about the medical institution, departments, and administration, day, date, and hour.
 - b. Patient full name, age gender and patient identification number
 - c. The nature of the patient's condition; including significant medical background.
 - d. The proposed treatment, possible treatment alternatives, including no treatment.
 - e. The benefits, outcomes as well as the possible significant risks of the proposed surgery/treatment or procedure
 - f. The individuals who will be providing treatment and the role of everyone in the proposed the proposed surgery/treatment or procedure.
 - g. Name and signature of the proceduralist, anesthetist and witness or interpreter.
 - h. Name, ID card number and the relationship of the person giving the consent to the patient.
 - i. The patient/legal representative should be given the opportunity to ask questions and receive additional information as requested.

5.7 Obtaining Informed Consent

- i. The person who is performing the procedure or any member in his/her team is responsible to obtain informed consent, after providing detailed information.
- ii. Informed consent is discussed with the patient by the physician; with verbal discussion and/or supplemented through written additions that give further information relevant to the patient's condition, considering the main elements of informed consent.
- iii. If the patient is able to act on his/her own to agree or refuse a particular surgery/treatment or procedure, then his/her decision shall be respected. In case a patient/legal authorized representative refuses the proposed surgery/treatment or procedure, it is the responsibility of the treating/performing physician to make reasonable attempts to explain the risks involved in not managing the condition.
- iv. If the patient cannot sign his/her name; then a thumb mark is acceptable documentation for the agreement.
- v. The medical staff witnessing the process of informed consent signs as a witness.
- vi. If an interpreter issued in the process, the interpreter must sign in the area designated for that.

5.8 Documentation

- i. Ministry of Health approved consent form (available in Al-Shifa 3+) must be completed on all cases.
 - ii. A consent note must be documented in the patient record (Al-Shifa3+) after explaining to the patient or/and legal representative and filling up the consent form. It should include the following information:
 - a) Nature of the patient's condition, diagnosis, and treatment/procedures to be performed.
 - b) Expected benefits, potential complications, and risks of the proposed surgery/treatment or procedure.
 - c) Alternatives of a given procedure or intervention.
 - d) Risks and benefits of alternatives.
 - e) Possible consequences of refusing not to intervene such as treatment/procedure/testing etc.
 - f) The proposed surgery/treatment or procedure cost if applicable. (Private institutions or Government institutions - GOV or Non-

GOV employee expatriates *Based on civil service council decision No.13/2016).

- g) The validity of informed consent must be specified.
- h) Assessment of the patient's understanding of the above elements.
- i) Person/s involved in the procedure.
- iii. The Consent form shall be written in comprehensible English and Arabic language. If necessary and feasible, the consent form shall be provided in a translated version in the native language of the patient.

5.9 Exceptions

- i. There are common exceptions to the informed consent rule in certain circumstances as the following:
 - i. Incapacitated patient and incompetence to make decisions to (agree or refuse) for the proposed tests, procedures or treatment.
 - ii. Life-threatening emergencies with inadequate time to obtain consent, in which curative care is immediately necessary to prevent a severe deterioration or irreversible harm. If the patient is unable to provide consent and no legal authorized representative is available, the treating/performing physician shall use his clinical judgment and accordingly proceed with the appropriate surgery/treatment or procedure without consent; supported by another senior physician in the same field.
 - iii. Voluntary waived consent.
 - iv. Intoxication
 - v. Other similar conditions
 - vi. The patient's inability to consent and lack of a legal representative must be clearly documented in the patient's medical record and signed by two physicians, one of whom shall be the treating Physician/Dentist/Proceduralist or any other treating healthcare providers

6 Responsibility

Duties and Obligations of Healthcare Providers on Obtaining the Informed Consent:

6.1 The treating physician or proceduralist:

- i. Disclose accurate and reliable information relevant to the patient's condition and obtain the patient's informed consent. Regardless of the type of consent, a clear explanation of the proposed surgery/treatment or procedure shall be given to the patient/legal representative.
- ii. The signed informed consent form should be kept in the patient's medical record in accordance with the health care institution's standards and procedures. Upon the patient/legal representative's request, a hard copy of the signed informed consent form shall be provided to the patient/legal representative.
- iii. All matters related to the informed consent shall be documented in the patient's medical record.

6.2 The nurse involved in the patient's care

- i. Verify with the patient that consent has been obtained by the physician/proceduralist prior to the proposed surgery/treatment or procedure and must document that in the medical record.
- ii. In the event when the nurse determined that the informed consent has not been obtained or documented, the nurse has to contact the physician to complete the consent process, talk to the patient, and/or provide specific documentation of the informed process which has previously taken place.

6.3 Operating /Procedure room supervisor:

Verifies that:

- A. Informed Consent has been obtained by the anesthesiologist and/or physician before the induction of the anesthesia and/or performing the procedure.
- B. The witness signed informed consent as well as the patient.
- C. The patient is willing to proceed with the proposed surgery/treatment or procedure and did not change his/her mind.

6.4 Check for validity of the informed consent against the following criteria:

- A. Information has been provided to the patient prior to the proposed surgery/treatment or procedure.
- B. Has been explained to the patient by the anesthesiologist and the proceduralist.
- C. The consent form has been filled in with the necessary information.
- D. All signatures required have been obtained.

6.5 Assures that the patient has been informed and can:

- A. State the name of the physician or other practitioner who has primary responsibility for the patient's care (Oman National Patient Rights & Responsibilities Declaration – phrase 3).
- B. Identify the professional status of individuals responsible for authorizing and performing the proposed surgery/treatment or procedure

6.6 Patient/ Legal authorized representative:

- The patient/legal authorized representative must make sure that they received all the information required before signing on the informed consent.
- The patient/legal authorized representative has the right to change their mind prior to the initiation of the proposed surgery/treatment or procedure.

7. Document History and Version Control

Version	Description	Review Date
1	Initial Release	2017
2	Second release	2022

8. Related Documents:

1. P&P of Patient Identification (MOH/DGQAC/004/Vers1.0)
2. P&P of Safe Surgery, Preoperative Communications (MOH/DGQAC/006/Vers1.0)

3. P&P of
SafeSurgery, Ensure correct patient, procedure, and correct site for invasive/surgical
procedures (MOH/DGQAC/007/Vers1.0)

9. References:

Title of book/ journal/ articles/ Website	Author	Year of publication	Page
Policy and Procedures of Leave Against Medical Advice (LAMA)	DGSMC		4, 5 & 16
http://www.dar.emory.edu/vetcare/surg_def_initiations.php			
National informed consent policy MOH, Qatar			
Suadi Guidelines for Informed Consent			

Appendix 1

Patient's Consent Form for Case Report استمارة الموافقة على نشر حالة طبية لغرض التعليم الطبي المستمر MOH/DGQAC /P&P /000/FRM 01/Vers.01

• To be completed by the treating practitioner:

• يستكمل من قبل الطبيب المعالج:

The provisional title of the case report:-----

عنوان الحالة:-----

Description of article, content, or photograph (the "Material"):

وصف الحالة-صورة (تحديد العضو)

The treating practitioner, who has explained and administered the form to the patient/ staff number:

اسم الطبيب او المعالج، الذي قام بشرح الاستمارة وتقديمها للمريض/الرقم الوظيفي:

Designation /signature:

المسمى الوظيفي / التوقيع:

• **To be completed by the patient:**

As per the attending physician's explanation, I am not opposed to publishing the above case

• **I understand that:**

1. My participation in this report is not compulsory
2. My medical records (personal health information) will be accessed according to the information provided in this form.
3. My identity will not be reveal for everyone, and in the event that a person can discover my identity, it will not be revealed by anyone in the health institution
4. Among the possible use of the pictures taken will be for publication in either online journals or printed journal editions.
5. The hospital has the right to release the pictures as and when needed on websites or printed without receiving any funds in return
6. The Material may be edited, modified, and retouched.

• **يستكمل من قبل المريض:**

لا امانع من نشر الحالة المذكورة أعلاه بناءً ما تم شرحه لي من قبل الطبيب المعالج

• **انا افهم انه:**

1. تم إبلاغي بأنني لست مضطراً للمشاركة في تقرير هذه الحالة
2. سيتم الوصول إلى معلوماتي الصحية الشخصية من (السجل الطبي) كما هو موضح في هذا النموذج
3. لن يتم نشر اسمي مع المادة من قبل المؤسسة الصحية وسوف يتم الحفاظ على سرية هويتي. وأدرك أنه من الممكن أن يتعرف على شخص ما من الصور المنشورة / أو المحتوى المصاحب.
4. قد يشمل استخدام المواد المتعلقة بي، على سبيل المثال لا الحصر، النشر في الإصدارات المطبوعة والإلكترونية من منشورات المجتمع، وعلى مواقع الويب، وفي الإصدارات المرخصة أو المعاد طباعتها (بما في ذلك الإصدارات باللغات الأجنبية)
5. أمانح وأعفي المؤسسة الصحية من جميع الحقوق والملكية والمصالح التي قد تكون لدي في المادة. وأعلم أنني لن أتلقى أي مبالغ تتعلق بذلك وأتنازل عن جميع المطالب (الشخصية، والمادية، والمعنوية).
6. قد تطرأ بعض التعديلات على المادة المحررة.

Patient name: -----

Signed: -----

If you are not the patient, what is your relationship to him/her:

Date:----- Time :-----

For Witness:-----

Signature: -----

Staff no.:-----

Date:----- Time: -----

اسم المريض: -----

التوقيع:-----

إذا تمت الموافقة من غير المريض، صلة القرابة -----

الاسم -----

تاريخ:----- الوقت:----- التوقيع:-----

اسم الشاهد: -----

التوقيع:-----

الرقم الوظيفي:-----

تاريخ:----- الوقت:-----

Note for the treating practitioner:

1. This form to be attached with official letter to the in charge of the health institution
2. copy to be sent quality management and patient safety department for information

ملحوظة للطبيب المعالج:

1. ترافق هذه الاستمارة برسالة رسمية إلى مسئول المؤسسة الصحية للحصول على الموافقة
2. نسخ إلى قسم إدارة الجودة وسلامة المرضى للعلم

Appendix 2: Assessment Criteria for Decision-Making Capacity

Criterion	Patient's Task	Physician's Assessment Approach	Questions for Clinical Assessment *patient's responses to these questions don't need to be verbal	Comments

Communicate a choice	Clearly indicate preferred treatment option	Ask patient to indicate treatment choice	- Have you decided whether to follow your doctor's (or my) recommendation for treatment? - Can you tell me what the decision is? - (if no decision) what is making it hard for you to decide?	Frequent reversals of choice because of psychiatric or neurologic conditions may indicate lack of capacity
Understand the relevant information	Grasp the fundamental meaning of information communicated by physician	Encourage patient to paraphrase disclosed information regarding medical condition and treatment	Please tell me in your own words what your doctor (or I) told you about: - The problem with your health now. - The recommended treatment. - The possible benefits and risks (or discomforts) of the treatment. - Any alternative treatments and their risks and benefits. - The risks and benefits of no treatment.	Information to be understood includes nature of patient's condition, nature and purpose of proposed treatment, possible benefits and risks of that treatment, and alternative approaches including no treatment) and their benefits and risks
Appreciate the situation and its consequences	Acknowledge medical condition and likely consequences of treatment	Ask patient to describe views of medical condition, proposed treatment, and	- What do you believe is wrong with your health now? - Do you believe that	Courts have recognized that patients who do not acknowledge their illnesses (often referred to as "lack of insight") cannot make valid

	options	likely outcomes	you need some kind of treatment? - What is treatment likely to do for you? - What makes you believe it will have that effect? - What do you believe will happen if you are not treated? - What do you think your doctor has (or I have) recommended this treatment?	decisions about treatment. Delusions or pathologic levels of distortion or denial are the most common causes of impairment.
Reason about treatment options	Engage in a rational process of manipulating the relevant information	Ask patient to compare treatment options and consequences and to offer reasons for selection of option	- How did you decide to accept or reject the recommended treatment? - What makes (chosen option) better than (alternative option)?	The criterion focuses on the process by which a decision is reached, not the outcome of the patient's choice, since patients have the right to make "unreasonable" choices

Appendix 3: The Informed Consent, which should meet the following criteria, was determined:

Type of consent	Duration of consent
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Informed consent	- Valid from the time signed by the patient or the next of kin/legal guardian to the time of the procedures/treatments is performed - The validity should not exceed ninety (90) days from the date of signature if the patient condition has not changed.
Anesthesia	
Use of blood and blood products	- Valid per episode of administering or order - Patient who needs multiple transfusions the consent is valid up to one (1) year if there is no change in the patient's condition
Chemotherapy	Valid for the whole course of treatment unless there is a change in the plan of treatment due to altered blood chemistry.
Radiation Therapy	
Dialysis	
Magnetic Resonance Imaging (MRI)	- Valid for two (2) years. - Patient will need to be evaluated by the anesthesia team within (4-6) weeks prior to the MRI.